

Women's Specialists of Clear Lake

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Labor Induction

When and why labor is started with medications or other methods — what to expect during the process

WHAT IS LABOR INDUCTION?

- The use of **medications or other methods** to bring on labor
- Goal is to stimulate uterine contractions for a **vaginal birth**
- May be recommended if **mother or baby's health** is at risk
- **Elective induction** is for nonmedical reasons and should not happen before **39 weeks**
- Medical reasons include high blood pressure, diabetes, or concerns about the baby

IS YOUR CERVIX READY? (BISHOP SCORE)

- Before labor, the cervix naturally softens, thins, and opens
- The **Bishop score** (0–13) rates how ready your cervix is for labor
- A score **less than 6** means your cervix may not yet be ready
- Lower scores mean cervical ripening may be needed first
- Higher scores are associated with more successful induction

CERVICAL RIPENING METHODS

- **Prostaglandins** — medications inserted vaginally or taken by mouth to soften the cervix
- Some prostaglandins are **avoided after prior cesarean** due to uterine rupture risk
- **Foley balloon catheter** — a small tube with an inflatable balloon to widen the cervix
- **Laminaria** — a natural substance that absorbs water and gently expands the cervix
- **Membrane stripping** — sweeping a finger along the amniotic sac to release natural prostaglandins

INDUCTION METHODS

- **Oxytocin (Pitocin)** — IV hormone that causes contractions; usually starts within **30 minutes**
- **Amniotomy** — breaking the water with a small instrument
- Amniotomy is done when the cervix is **dilated and thinned** and baby's head is low
- Most women go into labor **within hours** of water breaking
- Multiple methods may be used together for best results

RISKS OF INDUCTION

- **Too many contractions** (uterine overstimulation) can affect the baby's heart rate
- **Infection** of the mother or baby, especially after water breaks
- **Uterine rupture** — rare but serious, particularly with prior cesarean
- **Umbilical cord problems**
- **Increased chance of cesarean**, especially in first-time mothers with an unripe cervix

WHAT TO EXPECT

- ✓ Induction can take **hours to days** — especially if the cervix is not yet ripe
- ✓ You'll be monitored closely — contractions and baby's heart rate are watched continuously
- ✓ Pain management options (IV, epidural) are available when you need them
- ✓ Sometimes induction does not work and **cesarean delivery** may be needed
- ✓ Bring comfort items, snacks for your partner, and patience

⚠️ CALL (281) 557-0300 FOR EMERGENCIES OR IF YOU EXPERIENCE

- ⚠️ Heavy vaginal bleeding (more than spotting)
- ⚠️ Severe or constant abdominal pain
- ⚠️ Sudden gush of fluid (water breaking)
- ⚠️ Decreased or no fetal movement
- ⚠️ Severe headache or vision changes
- ⚠️ Signs of labor: regular, painful contractions

You're Almost There — We're With You Every Step

Labor induction is a safe, common way to help your baby arrive when needed. Our team will explain each step, answer your questions, and support you throughout the process. Don't hesitate to ask anything — we want you to feel informed and confident!